CADENCE

June 13, 2025

Stephanie Carlton
Deputy Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

Dr. Thomas Keane
Assistant Secretary for Technology Policy
Office of the Secretary
Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

SUBMITTED ELECTRONICALLY VIA: https://www.regulations.gov

RE: Request for Information (RFI); Health Technology Ecosystem, CMS-0042-NC

Dear Deputy Administrator Carlton and Assistant Secretary Keane:

At Cadence, we're on the front lines of a new era for healthcare where personalized, technology-driven care empowers patients to take control of their health and dramatically reduces costs for Medicare. We appreciate the opportunity to respond to this RFI.

Today, Remote Physiologic Monitoring (RPM) and Advanced Primary Care Management (APCM) are already proving what's possible: better outcomes, lower costs, and more engaged patients, including in rural communities. RPM – an accomplishment of the first Trump Administration – alone has demonstrated cost savings of $1,300 per patient per year, driven by 27% fewer hospitalizations.[1] To accelerate this transformation, we urge CMS to:

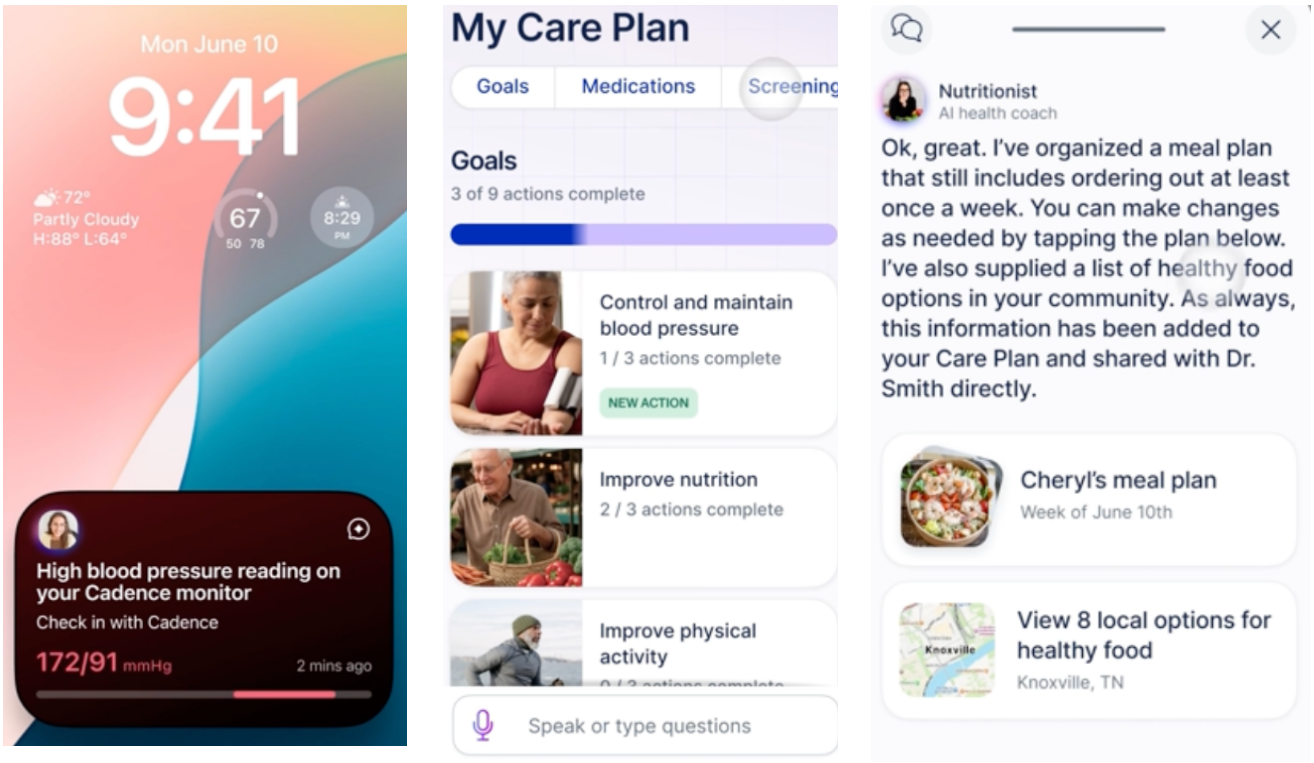
- Unleash patient-centered innovation by (a) eliminating unnecessary time-based requirements that limit the appropriate use of RPM and artificial intelligence (AI) enabled care and (b) expanding access to technology-driven services like APCM.
- Modernize RPM reimbursement to capture the value of software and related capabilities that create actionable, interoperable data, improve clinician workflows, and will soon deliver AI-driven clinical recommendations.

These changes should be balanced by raising the bar to protect Medicare from low-value utilization, through steps such as requiring 24/7 clinical support and leveraging full electronic medical record integration.

The future of healthcare doesn't need to be decades away. We are building it now. A demo video of Cadence's remote care delivery system that includes both current and future-state functionality can be found here (see Figure 1 for screenshots).

[1] Altchek C., Feldman D., Fudim M., Presentation, American Heart Association Scientific Sessions, November 2024, "Implementing Guidelines, Improving Outcomes & Lowering Cost with Remote Patient Care" (p. 15), https://ahahealthtech.org/wp-content/uploads/2024/12/Altchek_Health-Tech-Revolution.pdf.

Figure 1: Cadence's vision for remote patient care for Medicare beneficiaries.



A. About Cadence

Cadence delivers RPM, APCM, and Chronic Care Management (CCM) to over 50,000 patients nationwide, many of whom live in rural and underserved areas. Our focus is on using cutting-edge technology to manage and treat chronic conditions such as congestive heart failure, hypertension, and type 2 diabetes. We work alongside the most innovative health systems to give patients 24/7 access to our clinical team, identifying and treating issues before they escalate.

Our recent innovations showcase what's possible when you combine clinical excellence with technology-enabled care:

- **AI-Assisted Coaching Delivers Highly Personalized Care at Scale.** Our clinical team conducts over 12,000 lifestyle and wellness coaching visits per month. An AI-powered coaching co-pilot provides enhanced personalization, surfacing insights from the patient's medical and social history, and supports clinicians in crafting actionable goals that motivate and inspire patients. This blend of human judgment and AI insight transforms the patient experience, delivering tailored, high-touch care at scale without compromising safety or quality, thanks to human-in-the-loop oversight from our clinical team.
- **Patient Coordinator Agent Enables Consistency in Workflows and Dramatic Time Savings.** To facilitate seamless support for thousands of sites of care, our Patient Coordinator Agent

automates daily administrative tasks in multiple electronic medical records (EMRs), eliminating work previously managed by humans (2-5 minutes per task). Scheduling follow-ups, sending lab reminders, and the coordination of work among providers is now handled through automation.

- **Proactive Titrations via RPM Permit Faster, More Targeted Interventions.** Cadence's Proactive Titrations engine scans patients' charts in between in-office visits to surface patients that meet criteria for a medication titration algorithmically and automatically creates a list for consideration by Cadence nurse practitioners. This asynchronous model enables faster, more targeted interventions, which recent data show has resulted in an additional 4,170 (23%) Cadence patients achieving hypertension control (<130/80 mmHg) at 6 months following enrollment.
- **Proactive Care via APCM Continuously Assesses Gaps and Prompts the Next Best Action Across 30+ Longitudinal Care Goals.** What if your doctor could watch you every day, not every six months? Cadence's Proactive Care Model surfaces timely check-in tasks and reminders to reduce gaps in care, such as scheduling breast cancer screenings and colonoscopies. The program prompts patients as they progress toward achieving personalized care goals, including by encouraging healthy eating and connecting patients to community resources for social needs.

The first Trump Administration catalyzed this tremendous opportunity for technology-enabled care to reduce Medicare expenditures and help seniors. We've built Cadence on the foundation of those reforms and the results are in. Technology-enabled care is working – improving outcomes, lowering costs, and expanding access for the seniors who need it most. We are ready to take it to the next level.

But the barriers we face today, including Medicare's outdated valuation of technology-enabled care and rules discouraging use of AI, threaten innovation. It's time for CMS to clear the path for what's next.

B. Unleash patient-centered innovation by removing barriers to AI scaling of services.

Cadence's "nurse in your pocket" assistant keeps patients healthy, at home, through day-to-day support, easily accessible lifestyle coaching, and timely interventions. Real-world case studies demonstrate the crucial role Cadence's technology platform plays in empowering patients to follow their care plan and access needed resources: Patients are improving their medication adherence through better understanding of each prescription and its purpose, scheduling screenings timely to catch issues before they become severe, and more seamlessly coordinating care day-to-day with caregivers. Core capabilities include:

- A personalized care plan built in partnership with the patient's existing PCP
- On-demand health coaching
- Timeline reminders and notifications to drive accountability and adherence
- Remote monitoring of patient vitals
- Care navigation services, to ensure timely access to providers, specialists, and community resources
- Tracking of health gaps and closures, such as completion of any ordered screenings, tests, and/or vaccinations
- Connectivity to the full EMR
- An engaging user interface that is accessible to both seniors and their caregivers (i.e., app, voice, and text)

Peer-reviewed analysis of Cadence's remote care program demonstrates significant improvement in clinical outcomes alongside substantial cost savings in the form of reduced hospital and post-hospital discharge spending. For heart failure patients, adoption of Cadence's RPM plus medication optimization program led to three times more patients taking guideline-directed medical therapy and resulted in monthly savings on average of over $1,000 per patient.[2] Published data also show that the Cadence program led to two times more patients achieving blood pressure goals as compared to patients not on the program.[3] These positive clinical outcomes extend, and apply equally, to patients in rural and non-rural areas.[4]

The potential cost savings of this program to Medicare are tremendous. A cost and utilization analysis, which included 5,872 patients enrolled in Cadence's RPM program compared to 11,449 patients in a propensity-score matched control group, demonstrated annual total savings of $1,308 per patient across all chronic disease programs (heart failure, hypertension, and type 2 diabetes).[5] Cost savings were primarily driven by a 27% reduction in hospital admissions – specifically, reduced hospitalizations for heart failure and stroke.[6]

Enabling this solution at scale within Medicare requires CMS to take more steps to encourage technology-enabled care such as RPM and APCM services. Steps that would encourage the industry to fully meet patient needs include:

1. **Eliminating time-based requirements for team-based care management services like RPM.** The requirement of a minimum of 20 minutes of clinical staff time to provide the service restricts the use of (and willingness of companies and providers to invest in) technology that would deliver more personalized and efficient care. These services save money for the Medicare program, and it is in Medicare's interest to encourage wider availability.
2. **Expanding access to APCM services by removing the requirement that they be offered only by providers participating in an ACO or specified value pathway.** The APCM approach is a critical downpayment on the vision of moving beyond time-based care requirements. This vision can be more fully realized by expanding access beyond providers participating in a Shared Savings Program ACO or certain Innovation Center models (ACO REACH, Making Care Primary, etc.).

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[2] Feldman, D., et al. Leveraging Remote Patient Monitoring to Effectively Put the Heart Failure Guidelines to Practice. Journal of Cardiac Failure. Sept. 2024; Vol. 30 (Issue 9, pp. 1166-1169). https://doi.org/10.1016/j.cardfail.2024.04.018.

[3] Feldman D., Fudim M., et al. Abstract 12950: A nationwide remote patient intervention hypertension program: Can remote patient monitoring and a multi-disciplinary team of clinicians improve blood pressure control? Circulation. 2023;148 (Supp. 1): 12950. doi:10.1161/circ.148.suppl_1.12950. (Goal defined as <130/80 mmHg.)

[4] Emma Beavins, "Cadence and Lifepoint: Remote monitoring shows promise to deliver better health for rural patients," Fierce Healthcare (Sept. 11, 2024), https://www.fiercehealthcare.com/digital-health/cadence-and-lifepoint-rural-patients-surpass-urban-outcomes-through-rpm. (Note: Peer-reviewed publication demonstrating similarly positive clinical outcomes for rural and non-rural patients participating in RPM services is forthcoming, expected late June 2025.)

[5] Altchek C., Feldman D., Fudim M., Presentation, American Heart Association Scientific Sessions, November 2024, "Implementing Guidelines, Improving Outcomes & Lowering Cost with Remote Patient Care" (p. 15), https://ahahealthtech.org/wp-content/uploads/2024/12/Altchek_Health-Tech-Revolution.pdf (p-value=0.0026).

[6] Id. (p-value=0.0002).

This requirement limits the ability for APCM to be transformational and artificially restricts patients from accessing services where their provider cannot participate in such models. At a minimum, to eliminate uncertainty, CMS should allow providers to continue to provide APCM services for an extended period even if a particular value-based model in which they are participating is eliminated and thereby expand eligibility for APCM.

3. **Simplifying consent requirements across care management programs.** Patients should be able to consent just once to an overarching care management program that encompasses potentially overlapping services such as Principal Care Management, CCM, APCM, and RPM. Providers need the flexibility to meet a patient's needs via modular services without the administrative burden of obtaining consent multiple times.

C. Modernize RPM reimbursement to capture the value of software and related capabilities.

Technology-enabled services must be reimbursed through both fee-for-service and value-based mechanisms to encourage broad development and adoption. We urge CMS to invest in RPM and APCM by introducing codes that pay for the software underpinning these services, including technology platform interoperability and maintenance.

It is extremely resource intensive to create and maintain a high-quality, technology-driven remote care delivery system. For example, Cadence syncs patient data from its software to the EMR to ensure information is captured in the patient's chart and available at the point of care. We also staff a team to improve EMR integrations, which are far from standardized in the U.S. today, and employ full-time software engineers who design and engineer improvements, address software issues, and ensure the security of patient information. Cadence ensures the uptime of services, including connectivity of tens of thousands of cellular-enabled devices, to Cadence's software platform, and to dozens of EMRs, 24 hours a day, seven days a week.

Against this backdrop, we urge you to create a way to pay for RPM and APCM software. Given the constraints of the current practice expense methodology, CMS should use its authority to create new codes that capture the costs of software required to perform technology-enabled services like RPM and APCM. As part of this effort, CMS should consider requiring full EMR integration. We also urge CMS to explicitly promote the use of technology and AI in rulemakings, including for administrative and non-clinical tasks relevant to primary care and chronic condition management. Monitoring alone is no longer enough; the health care needs of Medicare beneficiaries require that this monitoring be tied to specific actions regarding usage of patient data (e.g., providing 24/7 clinical support) and AI capabilities such as development and tracking of highly personalized care goals.

D. Encourage performance measurement through timely access to Medicare claims data.

Finally, we urge Medicare to hold digital health solutions accountable, both in terms of delivering clinical outcomes and cutting costs. Physicians are open to using technology, including AI, in a clinical setting provided that the solutions are clinically safe and effective. CMS plays a key role in ensuring physician have access to such data.

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We support the use of CMS data to validate the patient outcomes and cost savings that we know exist from our own data. We encourage CMS to further leverage its tools to enable transparency regarding clinical efficacy, such as through analyses of claims data, which may be helpful in measuring cost and utilization effects of digital health solutions year-over-year. We would welcome the opportunity to work with CMS to determine how best to encourage performance measurement through the available tools as well as steps to permit the validation of robust private sector analysis and outcomes against CMS data.

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Thank you for your consideration. Providing American seniors with access to world-class digital tools aimed at prevention and management of chronic disease is a national imperative. We welcome the opportunity to engage with you in greater depth on the feedback presented above. Should you have any questions, require additional information, or wish to meet to review data supporting our comments, please contact Meryl Holt, Head of Legal, at meryl@cadencerpm.com.

Sincerely,

Christopher Altchek
Founder & CEO, Cadence